CONFIDENTIAL PATIENT INTAKE NOTE (FICTIONAL - DEMO DATA ONLY)

Patient: Eleanor Whitfield MRN: DEMO-10258
Age: 72 Sex: Female Date: 2026-10-24
Location: Springfield, MA

CHIEF COMPLAINT
Shortness of breath and ankle swelling, worse when lying flat.

HISTORY OF PRESENT ILLNESS
72-year-old woman with heart failure with reduced ejection fraction (HFrEF, EF 35%) and chronic kidney disease stage 3, presenting with progressive orthopnea, bilateral lower-extremity edema, and a 6-pound weight gain over one week.

ACTIVE DIAGNOSES / CONDITIONS
- Heart failure with reduced ejection fraction (EF 35%)
- Chronic kidney disease, stage 3
- Atrial fibrillation
- Type 2 diabetes mellitus

SYMPTOMS
Orthopnea, Lower-extremity edema, Fatigue, Palpitations.

CURRENT MEDICATIONS
- Furosemide 40 mg, twice daily (active)
- Carvedilol 12.5 mg, twice daily (active)
- Sacubitril/valsartan 49/51 mg, twice daily (active)
- Apixaban 5 mg, twice daily (active)
- Empagliflozin 10 mg, once daily (active)

PREVIOUS TREATMENTS
- Cardiac catheterization (2024, no intervention)
- Cardioversion for atrial fibrillation (2023)

ALLERGIES
Sulfa drugs (hives).

LABS / MEASUREMENTS
- NT-proBNP: 2450 pg/mL (2026-10-19)
- Ejection fraction: 35 % (2026-09-14)
- eGFR: 48 mL/min/1.73m2 (2026-10-19)
- Potassium: 4.6 mmol/L (2026-10-19)
- HbA1c: 6.9 % (2026-10-02)

ASSESSMENT / PLAN
Evaluate for HFrEF device and drug trials. Optimize diuresis. Patient has limited mobility; prefers trial sites within 25 miles and no overnight stays.